THE MATERNITY METHOD

NCLEX CHEAT SHEET

Placenta Previa & Placental Abruption

Maternal & Newborn › Bleeding in Late Pregnancy

The Big Picture Two causes of **bleeding in the 2nd half of pregnancy**. **Previa** = the placenta sits **low**, over or near the cervix. **Abruption** = a normally placed placenta **pulls away early** from the uterine wall. Both can cause **hemorrhage**, and both put **baby's oxygen** at risk.

KNOW THE NUMBERS

0	≥ 30 mL/hr	2	< 34 wks	72 hrs
Vaginal exams with previa	**Urine output** goal	**Large-bore IVs**	**Betamethasone** if delivery may be early	**RhoGAM** if Rh−

MEMORY HACK

Previa = Painless · Abruption = Agony

Previa	Abruption	Previa	Abruption
bright red, painless, **soft** uterus	**dark red**, **painful**, **rigid** "board-like" uterus	**No** fingers in the vagina	Watch for **DIC** + hidden bleeding

Side by Side

	Placenta Previa	Placental Abruption
Bleeding	**Bright red**, may come and go; amount you see = amount lost	**Dark red**, or **hidden** behind the placenta; shock may be **worse** than bleeding you see
Pain	**Painless**	**Sudden, severe** belly or back pain
Uterus	**Soft**, relaxed, non-tender	**Rigid**, tender, high tone; fundal height may **rise**
Baby	Usually OK unless mom bleeds heavily; often **breech/transverse**	**Fetal distress** common: late decels, bradycardia, loss of variability
Risks	**Prior cesarean** or uterine surgery, prior previa, many births, older age, **smoking**, twins	**Hypertension/preeclampsia** (#1), **cocaine**, smoking, **trauma** (car crash, abuse), prior abruption, PROM
Diagnosis	**Ultrasound**	Clinical signs (ultrasound may miss it)
Delivery	**Cesarean** if it covers the cervix	Depends on severity; often urgent delivery

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Nursing Actions

Goal: **stabilize mom** + **watch baby**

STEP 1
Assess

- **VS** often; **continuous FHR** + contractions
- **Weigh pads** or count them (1 g ≈ 1 mL)
- Abruption: **mark the fundus**; rising height = **hidden bleeding**
- **I&O**: urine ≥ 30 mL/hr
- Ask about **trauma** and screen for **abuse** privately

STEP 2
Act

- **Left side**; O_2 per protocol
- **2 large-bore IVs**, fluids; CBC, **type & crossmatch**, coags
- **Previa: no vaginal exams, no sex, nothing in the vagina**
- **RhoGAM** if Rh−; **betamethasone** if < 34 wks

STEP 3
Prepare

- Blood products ready
- **Cesarean** setup
- **NICU** team for baby
- Watch postpartum for **PPH** (previa sits where the uterus contracts poorly)

DIC (Abruption Complication)

- Clotting factors get **used up** → bleeding everywhere
- **Oozing** from IV sites, gums, nose; **petechiae**, bruising
- **Labs:** ↓ platelets, ↓ **fibrinogen**, ↑ PT/PTT, ↑ D-dimer
- **Treat the cause** (deliver) + blood products

Previa at Home (Stable)

- **Pelvic rest:** no sex, tampons, douching
- **Activity** as ordered; avoid heavy lifting
- Come in **right away** for any bleeding or contractions
- Know the **fastest route** to the hospital
- Prior cesarean + previa → screen for **placenta accreta**

RED FLAGS · STOP AND CALL THE PROVIDER

Any 2nd/3rd-trimester bleeding · **Board-like** uterus · **Fast HR, low BP**, restlessness · **Late decels** or bradycardia · **Oozing** from IV sites

MUST-KNOW FOR THE NCLEX

- **Previa:** painless bright red bleeding, soft uterus → **never** do a vaginal exam.
- **Abruption:** painful, dark bleeding, rigid uterus, fetal distress → watch for **DIC** and hidden bleeding.
- **Hypertension** is the top risk for abruption; **prior cesarean** for previa.
- Priorities: **left side, 2 large IVs, labs, continuous FHR, prepare for cesarean.**

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QUICK CHECK · TEST YOURSELF

1. Painless, bright red bleeding at 32 weeks suggests _____.
2. A rigid, board-like abdomen with dark bleeding suggests _____.
3. Which assessment is contraindicated with previa?
4. A rising fundal height in abruption may mean _____ bleeding.
5. Oozing from IV sites + low fibrinogen = _____.

Answers: 1. placenta previa · 2. placental abruption · 3. vaginal (digital) exam · 4. hidden (concealed) · 5. DIC

For study use only. Always follow facility protocols, current guidelines, and provider orders.